Adhiparasakthi Hospitals

ஆதிபராசக்தி மருத்துவமனை

A clinical division of Melmaruvathur Adhiparasakthi Institute of Medical Sciences and Research

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NABL LAB · NABH ACCREDITED

DISCHARGE SUMMARY

Clinical summary of the hospital stay

Generated 10-11-2026

PATIENT

Mrs.SAGUNTHALA W/O CHELLPPAN

60 Y / FEMALE | Ward: FSW | Bed 229

UHID	IP NUMBER	ADMITTED ON	DISCHARGED ON
6213746	IP07034897	09-11-2026 / 01:22 AM	09-11-2026 / 07:55 PM

TREATING CONSULTANT **Dr. Swetha** — FSW

FINAL DIAGNOSIS

1. CHOLELITHAISIS WITH CHOLEDOCHOLITHIASIS
2. ACUTE PANCREATITIS
3. ACID PEPTIC DISEASE
4. THROBOCYTOPENIA

CHIEF COMPLAINT

C/O abdominal pain for 1 month

HISTORY OF PRESENTING ILLNESS

- The patient was apparently normal 1 month back, after which she developed abdominal pain
- H/O vomiting 3 episodes, non bilious , contains food particle x 1 day
- H/O of fever present 10 days back
- No H/O loose stools
- No H/O of constipation

PAST MEDICAL HISTORY

- H/o admission in outside hospital on 23/10/2026 and diagnosed as AFI with thrombocytopenia for which 2 unit FFP and platelet was transfused
- K/C/O APD for 3 years and on medication
- N/K/C/O Type 2 Diabetes Mellitus, Systemic Hypertension, Bronchial Asthma, Epilepsy or Thyroid disorders.

PAST SURGICAL HISTORY

No other significant surgical history.

PERSONAL HISTORY

- Mixed diet
- Normal sleep pattern
- No H/O smoking
- No H/O alcoholism

IMMUNIZATION HISTORY

2 doses of COVID-19 vaccination done.

GENERAL EXAMINATION

- Patient conscious, oriented and afebrile.
- No pallor, icterus, cyanosis, clubbing, lymphadenopathy or pedal oedema.

VITALS ON ADMISSION

- BP: 110/70 mmHg
- PR: 86/min
- RR: 16/min
- Temp: 98.2°F.

SYSTEMIC EXAMINATION

- CVS: S1, S2 heard.
- RS: Bilateral air entry present.
- CNS: No focal neurological deficit.

PER ABDOMEN EXAMINATION

Soft, diffuse tenderness present no guarding , no rigidity, bowel sounds present.

INVESTIGATIONS

Enclosed.

COURSE IN THE HOSPITAL

The patient came to the hospital with above mentioned complaints. All baseline investigations were done. CT of the abdomen on 8/11/2026 revealed Multiple choledocholithiasis with significant dilatation of CBD (13.5 mm) and peri-choledochal inflammatory fat stranding.Cholelithiasis with no evidence of cholecystitis.Bulky pancreas with peripancreatic fat stranding, suggestive of acute interstitial edematous pancreatitis likely biliary in etiology. Following which MRCP is done on 9/11/26 which revealed Cholelithiasis with choledocholithiasis.
Dilated common bile duct and mild dilatation of intrahepatic biliary radicles with smooth tapering at distal end of CBD - likely stricture, suggested ERCP correlation. Redemonstration of acute pancreatitis. Patient is being referred to higher center in view of medical gastroenterologist opinion.

TREATMENT GIVEN

- High fibre diet.
- Inj.ceftriaxone 1g IV 1-0-1
- Inj.metronidazole 500mg IV 1-1-1
- Inj. Pantoprazole 40 mg IV 1-0-1.
- Inj.Paracetomol 1g IV 1-1-1
- Inj.Buscopan 20mg in 100ml NS IV 1-0-1
- Inj.tramadol 100mg in 100ml NS
- Inj.ondensetron 4mg IV 1-0-1
- Vitals charting Q4hrly

CONDITION AT DISCHARGE

- Patient conscious, oriented, afebrile and haemodynamically stable.
- BP: 100/70 mmHg
- PR: 106 bpm
- RR: 16/min
- Temp: 97.6F
- CVS: S1, S2 heard.
- RS: Bilateral air entry present.
- CNS: No focal neurological deficit.
- P/A: Soft, Diffuse tenderness present , no guarding , no rigidity , no abdominal distension, bowel sounds present.
- REFERRED TO HIGHER CENTRE
- SIGNATURE & SEAL OF TREATING CONSULTANT
- EMERGENCY CONTACT: 044 27528528
- TOLL-FREE NO.: 1800 599 0999
- அவசர உதவிக்கு: 044 2752 8528
- கட்டணமில்லா எண்: 1800 599 0999

Prepared & Approved by Dr. Swetha

24/7 EMERGENCY HELPLINE: 044 2752 8528 | Toll-Free: **1800 599 0999**
அவசர உதவிக்கு: 044 2752 8528 | கட்டணமில்லா தொலைபேசி எண்: 1800 599 0999

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